SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Irinotecan · every three weeks

Advanced / metastatic colorectal cancer · NCCP 00213 · version 7 · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Schedule
Irinotecan on Day 1 every 21 days while beneficial and tolerable. There is no home chemotherapy pump. Blood counts are checked weekly, including between infusion visits.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Irinotecan

TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions and the contact plan. Explain early and delayed diarrhoea separately.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00213 v7.

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